PHASE 1: FOUNDATION & STABILIZATION

WEEK 1: WELCOME & UNDERSTANDING CSBD

What Is Compulsive Sexual Behavior Disorder?

Compulsive Sexual Behavior Disorder (CSBD) is recognized in the **ICD-11** (the World Health Organization's International Classification of Diseases, 11th Revision) as an **impulse control disorder**.

The official definition:

"A persistent pattern of failure to control intense, repetitive sexual impulses or urges, resulting in repetitive sexual behavior that causes marked distress or impairment in personal, family, social, educational, occupational, or other important areas of functioning."

Let's break that down into plain language:

Key features of CSBD include:

1. Repetitive sexual activities become a central focus of your life

- You spend significant time thinking about, planning, or engaging in sexual behavior
- Sexual behavior takes priority over other important activities
- Your life starts to revolve around opportunities for sexual activity

2. Numerous unsuccessful efforts to reduce or control the behavior

- You've tried to stop or cut back many times
- You've made promises to yourself or others
- Despite genuine effort and intention, the behavior continues
- You feel out of control

3. Continued behavior despite harmful consequences

- You keep engaging in the behavior even though it's damaging your life
- You're aware of negative consequences but continue anyway
- The short-term relief outweighs the long-term damage in the moment

4. Distress or impairment in important areas of functioning

- Relationships are damaged or destroyed
- Work performance suffers
- Financial problems arise
- Health is compromised
- Legal issues may occur
- Spiritual or moral distress is significant

How common is CSBD?

Research indicates that approximately **3-6% of the general population** meets criteria for CSBD. That means millions of people struggle with this. You are not alone. You are not uniquely broken.

Important: CSBD is a **clinical condition**, not a moral failure or a character flaw. It's not about being a "bad person" or lacking willpower. It's a recognized impulse control disorder that responds to treatment.

The Addiction Cycle

CSBD follows a predictable, repeating cycle. Understanding this cycle is the first critical step toward interrupting it.

Most men with CSBD can map their behavior onto this four-stage cycle:

THE FOUR STAGES:

1. PREOCCUPATION

This is where the cycle begins. Your mind becomes increasingly focused on sexual thoughts, fantasies, or planning.

What it looks like:

- Obsessive thoughts about sex, pornography, or specific sexual behaviors
- Mental planning: when, where, how you'll act out
- Fantasy and rehearsal in your mind
- Feeling "zoned out" or in a trance-like state
- Difficulty concentrating on work, conversation, or other tasks
- Increasing anxiety, restlessness, or agitation

What you might notice physically:

- Tension in your body
- Restlessness, inability to sit still
- Increased heart rate
- A sense of being "wound up"

2. RITUALIZATION

Ritualization is the bridge between thinking about the behavior and actually doing it. These are the preparatory steps that lead directly to acting out.

What it looks like:

- Opening your browser in incognito or private mode
- Driving toward a location where you typically act out
- Checking to see if you're alone or will have privacy
- Arranging your schedule to create opportunity
- "Just checking" certain websites, apps, or locations
- Ensuring you have time, money, or access
- Creating the conditions that enable the behavior

Why this stage is critical: For most men, once ritualization begins, the behavior becomes extremely difficult to stop. The rituals create momentum. Recognizing your specific rituals gives you an earlier intervention point.

3. ACTING OUT

This is the sexual behavior itself. What this looks like varies from person to person:

- Viewing pornography (often for hours)
- Compulsive masturbation (multiple times per day)
- Hiring sex workers or escorts
- Anonymous sexual encounters (apps, bathhouses, rest stops)
- Visiting strip clubs, massage parlors, or adult stores
- Excessive use of dating or hookup apps
- Voyeuristic behavior
- Exhibitionism

- Any other sexual behavior done compulsively despite negative consequences

What it feels like in the moment:

- Temporary relief from anxiety, stress, or tension
- Brief pleasure or excitement
- Numbness or escape from difficult emotions
- A sense of release (physical and emotional)
- Tunnel vision—nothing else matters, consequences disappear
- Dissociation from reality
- Time distortion (hours feel like minutes)

4. DESPAIR

After acting out, reality crashes back in. The shame cycle begins.

What it looks like:

- Intense shame and self-hatred
- Guilt about broken promises (to yourself, to God, to your partner)
- Fear of being discovered or exposed
- Promises to "never do it again"
- Feeling hopeless about the possibility of change
- Isolation and hiding from others
- Depression, anxiety, or emotional numbness
- Self-punishment or negative self-talk

The cruel trap: This despair creates the exact emotions (shame, anxiety, hopelessness, disconnection) that trigger the next cycle. You feel terrible, so you need escape, so you return to the very behavior that made you feel terrible in the first place.

THE SELF-PERPETUATING CYCLE

Here's why this cycle is so hard to break:

DESPAIR → Uncomfortable emotions → PREOCCUPATION → Relief through fantasy → RITUALIZATION → Momentum and arousal → ACTING OUT → Temporary relief → DESPAIR → [Cycle repeats]

The cycle feeds itself. Each stage creates the conditions for the next stage.

Breaking the cycle requires intervening EARLY—ideally in the preoccupation or ritualization stages, before you reach acting out. Once you're in the acting out stage, stopping requires enormous effort. Once you're in despair, you're already setting up the next cycle.

Your goal over the next 16 weeks: Learn to recognize where you are in the cycle and interrupt it before reaching the point of no return.

Understanding What This Is NOT

Before we go further, let's clear up common misconceptions about CSBD:

CSBD is NOT:

❌ **Simply having a high sex drive**

- Having frequent sexual desire is normal
- CSBD is about loss of control and negative consequences, not frequency alone

❌ **Enjoying sex or masturbation**

- Healthy sexuality includes pleasure
- The issue is compulsion, not enjoyment

❌ **Having sexual thoughts or fantasies**

- Sexual thoughts are normal and healthy
- CSBD involves obsessive, intrusive thoughts that interfere with functioning

❌ **Being interested in pornography**

- Many people view pornography without problems
- CSBD involves compulsive use despite harm

❌ **Any specific type of sexual behavior**

- The behavior itself doesn't define CSBD
- The issue is compulsivity and consequences, not what the behavior is

❌ **Being a "sex addict" with no hope of normal sexuality**

- This label can be stigmatizing and inaccurate
- The goal is healthy, values-aligned sexuality, not lifelong abstinence

CSBD IS:

✓ **Loss of control** over sexual behavior despite repeated attempts to stop

✓ **Continued behavior** despite significant negative consequences

✓ **Using sex as the primary coping mechanism** for uncomfortable emotions

✓ **Significant distress or life impairment** in relationships, work, health, finances, or other important areas

✓ **Failed attempts** to stop or reduce the behavior on your own

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How CSBD Differs from Healthy Sexuality

Healthy Sexuality	CSBD
Enhances life and relationships	Damages life and relationships
Rooted in choice and mutual consent	Rooted in compulsion and urge
Connected to personal values	Conflicts with personal values
Enhances intimacy and connection	Substitutes for or avoids intimacy
Integrated with other areas of life	Separate, hidden, or compartmentalized
Flexible and adaptive	Rigid, ritualized, escalating
Brings genuine satisfaction	Brings temporary relief followed by shame
Sex as one part of a full life	Sex as escape from life
Moderate time and energy investment	Excessive time and mental energy
Open and honest (with appropriate people)	Hidden, secretive, shame-based

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The Brain Science: Why This Happens

You're not weak-willed. You're not morally deficient. Your brain has formed powerful neural pathways that make this behavior feel automatic and nearly impossible to resist.

Understanding the neuroscience helps remove shame and clarifies what you're actually working with.

THE BRAIN IN CSBD:

PREFRONTAL CORTEX (The Brake)

Function:

- Executive function and decision-making
- Impulse control and delayed gratification
- Planning and consequence evaluation
- Rational thought

In CSBD:

- Weakened ability to override urges
- Gets bypassed when you're triggered
- The "brake" doesn't engage effectively
- Difficulty thinking clearly in high-risk moments

LIMBIC SYSTEM (The Gas Pedal)

Function:

- Reward, motivation, pleasure
- Emotional responses
- Memory formation (especially emotional memories)
- Drives behavior toward rewarding experiences

In CSBD:

- Hypersensitive to sexual cues and triggers
- Floods the system with "GO" signals
- Creates powerful urges that feel life-or-death urgent
- Learns and remembers patterns quickly

THE RESULT:

Your gas pedal (limbic system) is extra sensitive and your brake (prefrontal cortex) is compromised. When triggered, your brain floods with "GO" signals while your ability to think rationally and inhibit behavior is weakened.

This isn't an excuse—it's an explanation.

And most importantly: **These patterns can change with practice.** Neuroplasticity means your brain can form new pathways. You can strengthen your brake and reduce the sensitivity of your gas pedal. It takes time,

repetition, and consistent practice—but it's absolutely possible.

The Role of Shame vs. Guilt

Understanding the difference between shame and guilt is **critical** to recovery. Most men confuse these two emotions, and that confusion keeps them trapped.

GUILT: "I did something bad"

Focuses on: Behavior

The message: "I made a mistake. I acted against my values."

The result: Motivates change and repair

Can be resolved through: Making amends, changing behavior, learning from mistakes

Function in recovery: Helpful—Guilt is appropriate accountability that drives positive change

Example: "I looked at pornography last night, and that's not aligned with my values. I need to understand what triggered me and practice my coping skills."

SHAME: "I am bad"

Focuses on: Identity and self-worth

The message: "I'm fundamentally broken. I'm a bad person."

The result: Perpetuates destructive cycles, leads to hiding and isolation

Can be resolved through: Self-compassion, connection, challenging distorted beliefs

Function in recovery: Harmful—Shame drives the cycle and prevents healing

Example: "I'm disgusting. I'm a failure. I'll never change. No one would love me if they knew who I really am."

THE SHAME SPIRAL IN CSBD:

SHAME → Feeling fundamentally broken → ISOLATION → Hiding from others → SECRECY → Increased disconnection → NEED FOR ESCAPE → Acting out for relief → MORE SHAME → [Spiral continues]

Most men with CSBD are drowning in shame. Shame tells you that you're fundamentally damaged, which makes you want to hide, which increases isolation and disconnection, which increases the need for escape and relief, which triggers more sexual behavior, which creates more shame.

Shame is the fuel that keeps the fire burning.

Recovery requires moving from shame to accountability:

- You did something you're not proud of → That doesn't make you irredeemable
- You have a pattern that causes harm → That doesn't make you fundamentally broken
- You struggle with compulsive behavior → That makes you human, not worthless

You are not your behavior. You are a person with inherent worth who happens to struggle with compulsive sexual behavior. There's a difference.

Statistics & Normalization

Sometimes knowing you're not alone helps reduce shame. Here are some statistics:

- **3-6%** of the general population meets criteria for CSBD
- **88%** of men report viewing pornography at some point
- **76%** of men who seek treatment for CSBD report childhood trauma or adverse childhood experiences
- **Average age of first exposure** to pornography is now 11-12 years old
- **Most common co-occurring conditions:** Depression (40-50%), anxiety (30-40%), ADHD, substance use disorders

These statistics aren't excuses. They're context.

You're not uniquely broken. This is a widespread issue that affects millions of men. The patterns you've developed make sense given your history, your environment, and how your brain works.

And recovery is absolutely possible.

EXERCISE 1: MY STORY IN BRIEF

Take time to write out your story. You don't need to include every detail—just enough to acknowledge where you are and why you're here.

This is for you. No one else needs to see this unless you choose to share it.

Be honest. Be specific. Don't minimize.

What brought me to this workbook:

_____

_____

_____

_____

_____

How long I've been struggling with compulsive sexual behavior:

_____

_____

Types of sexual behavior I've engaged in compulsively (check all that apply):

- ☐ Pornography
- ☐ Compulsive masturbation
- ☐ Anonymous sexual encounters
- ☐ Affairs or infidelity
- ☐ Hiring sex workers/escorts
- ☐ Strip clubs
- ☐ Massage parlors
- ☐ Dating/hookup apps
- ☐ Voyeurism or exhibitionism
- ☐ Sexting or online sexual activity
- ☐ Other: _____

Consequences I've experienced (check all that apply):

- ☐ Damaged primary relationship or marriage
- ☐ Divorce or separation
- ☐ Loss of trust from partner
- ☐ Strained family relationships
- ☐ Social isolation or lost friendships
- ☐ Work performance issues
- ☐ Job loss
- ☐ Financial problems (spending on sex, lost income)
- ☐ Legal issues
- ☐ Health consequences (STIs, physical problems)
- ☐ Spiritual or moral distress
- ☐ Depression or suicidal thoughts
- ☐ Anxiety or panic

☐ Shame and self-hatred

☐ Other: _____

Describe the most significant consequence you've faced:

_____

_____

_____

_____

_____

What I hope to gain from this 16-week process:

_____

_____

_____

_____

_____

One fear I have about this process:

_____

_____

_____

One hope I have about this process:

_____

_____

_____

_____

DAILY SELF-MONITORING LOG

Self-monitoring is the foundation of self-awareness. And self-awareness is the foundation of change.

For the next 16 weeks, you will track your urges, triggers, and behaviors daily.

This isn't about judgment. It's about data. You cannot change patterns you don't recognize.

Why track?

- **Identify patterns** you can't see otherwise (time of day, emotional states, situational triggers)
- **Recognize early warning signs** before they become overwhelming urges

- **Measure progress** over time (you'll forget how far you've come without data)
- **Build awareness** between trigger and action (the pause that creates choice)
- **Hold yourself accountable** to yourself

How to use the log:

1. Fill it out at the end of each day

- Set a phone reminder for the same time every night
- It takes 3-5 minutes
- Make it part of your nighttime routine

2. Be brutally honest

- No one else needs to see this unless you choose to share it
- Lying to yourself keeps you stuck
- Track even "small" urges, not just intense ones

3. Don't skip days

- Even "good" days with no urges need to be logged
- Pattern recognition requires consistent data
- Missing days creates gaps in your awareness

4. Review weekly

- At the end of each week, look back at all 7 days
- What patterns do you notice?
- What triggered you most often?
- When were you most vulnerable?

DAILY MONITORING LOG - WEEK 1

(Complete one log per day for 7 days. Additional blank logs are provided in Appendix A for all 16 weeks.)

DAY 1

Date: _____

Day of week: _____

Overall stress level today (1-10): _____

Sleep last night (hours): _____

HALT Check: Was I experiencing any of these today? (Circle all that apply)

Hungry | **A**ngry | **L**onely | **T**ired

URGES EXPERIENCED TODAY:

Number of urges: _____

Average intensity (1-10): _____

- 1-3: Mild, easily manageable
- 4-6: Moderate, required effort to manage
- 7-8: Strong, difficult to manage
- 9-10: Overwhelming, felt almost impossible to resist

How long did urges last?

☐ A few minutes
☐ 15-30 minutes
☐ 1+ hour
☐ On and off all day

TRIGGERS I NOTICED (What happened right before the urge? Be specific):

_____

_____

_____

_____

DID I ENGAGE IN PROBLEMATIC SEXUAL BEHAVIOR TODAY?

☐ No
☐ Yes

If YES, describe briefly (What behavior? How long? What happened?):

_____

_____

_____

_____

COPING STRATEGIES I USED (if any):

_____

_____

_____

EMOTIONS I FELT TODAY (Circle all that apply):

Anxious | Angry | Sad | Lonely | Bored | Stressed | Ashamed | Frustrated | Peaceful | Happy | Excited | Numb | Overwhelmed | Hopeful | Other: _____

_____

WHAT I LEARNED TODAY:

_____

_____

_____

_____

DAY 2

Date: _____

Day of week: _____

Overall stress level today (1-10): _____

Sleep last night (hours): _____

HALT Check: (Circle all that apply)

Hungry | **A**ngry | **L**onely | **T**ired

URGES EXPERIENCED TODAY:

Number of urges: _____

Average intensity (1-10): _____

How long did urges last? _____

TRIGGERS I NOTICED:

_____

_____

_____

DID I ENGAGE IN PROBLEMATIC SEXUAL BEHAVIOR TODAY?

☐ No
☐ Yes

If YES, describe briefly:

_____

_____

COPING STRATEGIES I USED:

_____

_____

EMOTIONS I FELT TODAY (Circle all that apply):

Anxious | Angry | Sad | Lonely | Bored | Stressed | Ashamed | Frustrated | Peaceful | Happy | Excited | Numb | Overwhelmed | Hopeful | Other: _____

WHAT I LEARNED TODAY:

_____

_____

_____

DAY 3

Date: _____

Day of week: _____

Overall stress level today (1-10): _____

Sleep last night (hours): _____

HALT Check: (Circle all that apply)

Hungry | **A**ngry | **L**onely | **T**ired

URGES EXPERIENCED TODAY:

Number of urges: _____

Average intensity (1-10): _____

How long did urges last? _____

TRIGGERS I NOTICED:

_____

_____

_____

DID I ENGAGE IN PROBLEMATIC SEXUAL BEHAVIOR TODAY?

☐ No
☐ Yes

If YES, describe briefly:

_____

_____

COPING STRATEGIES I USED:

_____

_____

EMOTIONS I FELT TODAY (Circle all that apply):

Anxious | Angry | Sad | Lonely | Bored | Stressed | Ashamed | Frustrated | Peaceful | Happy | Excited | Numb | Overwhelmed | Hopeful | Other: _____

WHAT I LEARNED TODAY:

_____

_____

_____

DAY 4

Date: _____

Day of week: _____

Overall stress level today (1-10): _____

Sleep last night (hours): _____

HALT Check: (Circle all that apply)

Hungry | **A**ngry | **L**onely | **T**ired

URGES EXPERIENCED TODAY:

Number of urges: _____

Average intensity (1-10): _____

How long did urges last? _____

TRIGGERS I NOTICED:

_____

_____

_____

DID I ENGAGE IN PROBLEMATIC SEXUAL BEHAVIOR TODAY?

☐ No
☐ Yes

If YES, describe briefly:

_____

_____

COPING STRATEGIES I USED:

_____

_____

EMOTIONS I FELT TODAY (Circle all that apply):

Anxious | Angry | Sad | Lonely | Bored | Stressed | Ashamed | Frustrated | Peaceful | Happy | Excited | Numb | Overwhelmed | Hopeful | Other: _____

WHAT I LEARNED TODAY:

_____

_____

_____

DAY 5

Date: _____

Day of week: _____

Overall stress level today (1-10): _____

Sleep last night (hours): _____

HALT Check: (Circle all that apply)

Hungry | **A**ngry | **L**onely | **T**ired

URGES EXPERIENCED TODAY:

Number of urges: _____

Average intensity (1-10): _____

How long did urges last? _____

TRIGGERS I NOTICED:

_____

_____

_____

DID I ENGAGE IN PROBLEMATIC SEXUAL BEHAVIOR TODAY?

☐ No
☐ Yes

If YES, describe briefly:

_____

_____

COPING STRATEGIES I USED:

_____

_____

EMOTIONS I FELT TODAY (Circle all that apply):

Anxious | Angry | Sad | Lonely | Bored | Stressed | Ashamed | Frustrated | Peaceful | Happy | Excited | Numb | Overwhelmed | Hopeful | Other: _____

WHAT I LEARNED TODAY:

_____

_____

_____

DAY 6

Date: _____

Day of week: _____

Overall stress level today (1-10): _____

Sleep last night (hours): _____

HALT Check: (Circle all that apply)

Hungry | **A**ngry | **L**onely | **T**ired

URGES EXPERIENCED TODAY:

Number of urges: _____

Average intensity (1-10): _____

How long did urges last? _____

TRIGGERS I NOTICED:

_____

_____

_____

DID I ENGAGE IN PROBLEMATIC SEXUAL BEHAVIOR TODAY?

☐ No
☐ Yes

If YES, describe briefly:

_____

_____

COPING STRATEGIES I USED:

_____

_____

EMOTIONS I FELT TODAY (Circle all that apply):

Anxious | Angry | Sad | Lonely | Bored | Stressed | Ashamed | Frustrated | Peaceful | Happy | Excited | Numb | Overwhelmed | Hopeful | Other: _____

WHAT I LEARNED TODAY:

_____

_____

_____

DAY 7

Date: _____

Day of week: _____

Overall stress level today (1-10): _____

Sleep last night (hours): _____

HALT Check: (Circle all that apply)

Hungry | **A**ngry | **L**onely | **T**ired

URGES EXPERIENCED TODAY:

Number of urges: _____

Average intensity (1-10): _____

How long did urges last? _____

TRIGGERS I NOTICED:

_____

_____

_____

DID I ENGAGE IN PROBLEMATIC SEXUAL BEHAVIOR TODAY?

☐ No
☐ Yes

If YES, describe briefly:

_____

_____

COPING STRATEGIES I USED:

_____

_____

EMOTIONS I FELT TODAY (Circle all that apply):

Anxious | Angry | Sad | Lonely | Bored | Stressed | Ashamed | Frustrated | Peaceful | Happy | Excited | Numb | Overwhelmed | Hopeful | Other: _____

WHAT I LEARNED TODAY:

_____

_____

_____

WEEK 1 REVIEW: PATTERNS & INSIGHTS

At the end of Week 1, review all 7 days and answer these questions:

What patterns do I notice?

- Time of day when urges are strongest: _____
- Day(s) of the week when urges are strongest: _____
- Most common triggers this week: _____
- Most common emotions before urges: _____

What surprised me this week?

_____

_____

What was most difficult this week?

_____

_____

What did I do well this week?

_____

_____

_____

REFLECTION: WHAT DOES SEXUAL INTEGRITY MEAN TO ME?

Before we go further, take time to define what sexual integrity means to you personally.

This is YOUR definition, not anyone else's. Not your partner's. Not your church's. Not society's. Yours.

Don't overthink it. Just write what comes to mind. You'll refine this definition throughout the 16 weeks.

_____

When I think about sexual integrity, I think about:

_____

_____

_____

_____

Living with sexual integrity would mean:

Sexual integrity would look like this in my daily life:

The gap between where I am now and where I want to be:

One small step I can take this week toward greater integrity:

WEEK 1 HOMEWORK CHECKLIST

Before moving to Week 2, complete:

- [ ] Read all Week 1 material thoroughly
- [ ] Complete "My Story in Brief" exercise
- [ ] Fill out daily monitoring logs for all 7 days
- [ ] Complete Week 1 Review: Patterns & Insights
- [ ] Write reflection: "What Does Sexual Integrity Mean to Me?"
- [ ] Identify and take one small integrity-aligned action this week
- [ ] Set up daily reminder to complete monitoring logs

Notes for Week 1:

END OF WEEK 1